WholeMind
RESILIENCE

The Case for WholeMind Resilience

Evidence of Professional and System Need

A review of current UK sector evidence on stress, burnout, vicarious trauma and mental wellbeing among public-facing professional 'helper' roles; health, social care, councils, education, care/care homes, community care, blue light services and the VCSE sector. Prepared August 2026.

42.4% NHS staff felt unwell due to work-related stress in the last 12 months (NHS Staff Survey 2025)	**58%** of police officers describe themselves as emotionally exhausted or burnt out (National Police Wellbeing Survey 2025)	**76%** of teachers reported workplace stress in 2025 (Teacher Wellbeing Index)

£51bn annual UK cost of poor mental health to employers (Deloitte, 2024)	**£6.30** returned for every £1 spent on universal, preventative interventions — the highest of any intervention type measured	**95%** of 122 public service professionals polled by WholeMind Resilience report prolonged stress, exhaustion or burnout in the past year

27% of workers received no support at all on return from stress-related absence (Mental Health UK, 2026)	**17%** had a formal return-to-work plan put in place	**35% → 18%** fall in staff desire to quit when managers are trained for supportive conversations (GBC)

1. Why we did the research

WholeMind Resilience's public survey (see the companion report, The Case for WholeMind Resilience: Evidence of Community Need) gathered primary evidence directly from the Norfolk public to understand the self-reported need for mental health support in the community. This report for commissioners and services uses:

- published workforce survey and associated research from organisations to understand the need for mental health support within organisations including; health, social care (including domiciliary, residential and nursing care), local authorities, education, community care, blue light services and the VCSE sector.

- published NICE guidance and NHS standards to highlight how WholeMind Resilience can support organisations to help prevent, manage and mitigate the impact of mental health issues within organisations

An informal WholeMind LinkedIn poll asked public service professionals across the sectors above whether they, or someone on their team, had faced prolonged high stress, exhaustion, vicarious trauma or signs of burnout in the past year. Of 122 respondents, 95% answered yes. Although, the poll was an informal, self-selected survey, it was direct engagement with WholeMind's actual target audience and the result aligns with findings from the different organisation workforce surveys and research.

2. The scale of the problem

NHS

Two useful sources of information to understand the level of need within health organisations include the NHS Staff Survey reported by the NHS Alliance[1] and the NHS Sickness Absence Report.[2]

The autumn 2025 NHS Staff Survey shows that more than 42% of staff have felt unwell due to work-related stress and 30% feel burnt out because of their work. Despite not feeling well enough 56% of staff have gone into work. There appears to be a gap in how staff feel their health and wellbeing is supported by their organisation with only 54.8% of staff reporting that their organisation takes positive action on health and well-being.

The NHS Sickness Absence Report shows that in April 2026 5.0% of staff were absent due to being sick and that 29.2% of absence was due to anxiety, depression or psychiatric illness. Of the various NHS organisations, ambulance trusts had the highest sickness absence rates of 6.4%.

This highlights the need for additional accessible mental health related support for staff across the NHS.

Indicator	Figure	Source
Felt unwell due to work-related stress in the last 12 months	42.4%	NHS Staff Survey
Feel burnt out because of their work	~30% (almost 1 in 3)	NHS Staff Survey
Attended work in the last 3 months despite not feeling well enough	56.0%	NHS Staff Survey
My organisation takes positive action on health and well-being	54.8%	NHS Staff Survey

[1] https://thenhsalliance.org/resources/nhs-staff-survey-results-2025-what-you-need-to-know

[2] https://digital.nhs.uk/data-and-information/publications/statistical/nhs-sickness-absence-rates/april-2026

Overall NHS sickness absence rate (April 2026, most current available)	5.0%	NHS Sickness Absence Report
Ambulance Trusts sickness absence rate (April 2026, highest of any NHS organisation type)	6.4%	NHS Sickness Absence Report
Of all NHS sickness absence, attributed to anxiety, stress, depression or psychiatric illness	29.2%	NHS Sickness Absence Report

Social Care

Within social care there is a similar picture. The Care Workers Charity Wellbeing Report 2025[3] shows that a significant proportion of respondents reported experiencing stress, unhappiness, and burnout. For example, 42.1% of respondents agreed or strongly agreed that they had been feeling unhappy or depressed, and 34% said their job negatively affects their mental health.

Again, there appears to be a gap in access to health and well-being support with only 52.6% of respondents stating that their workplace provides adequate mental health resources. And, 32.9% of staff reported needing therapy because of their job but more than half of staff who needed therapy have not accessed therapy because they don't know where to seek support.

The KingsFund show that in 2024/25 the workforce vacancy rate in care providers was about 7% compared to 2.4% the wider economy[4] and while pay is one of the contributing factors, employees also value good working conditions such as those highlighted in skills for care[5]. These include ensuring staff are physically and mentally fit for work. By making working conditions as good as possible vacancy and turnover can be reduced and in turn increasing workforce experience and improving continuity.

Indicator	Figure	Source
Feeling unhappy or depressed	42.1%	The Care Worker's Charity Wellbeing Report
Say their job negatively affects their mental health	34.3%	The Care Worker's Charity Wellbeing Report

[3] https://www.thecareworkerscharity.org.uk/news/the-care-workers-charity-publishes-the-findings-of-its-care-worker-wellbeing-survey/
[4] https://www.kingsfund.org.uk/insight-and-analysis/long-reads/social-care-360-workforce-carers
[5] https://www.skillsforcare.org.uk/resources/documents/Recruitment-support/Retaining-your-staff/Secrets-of-Success/Recruitment-and-retention-secrets-of-success-report.pdf

Need therapy because of my job	32.9%	The Care Worker's Charity Wellbeing Report
have not sought therapy because they are unsure where to find appropriate support	17.1%	The Care Worker's Charity Wellbeing Report

Education

The Teacher Wellbeing Index 2025[6] highlights a workforce that is experiencing poor health and wellbeing. 76% of staff are stressed, 36% are at risk of probable clinical depression, 77% experience symptoms of poor mental health due to work and 40% of staff say that employees with mental health issues are not well supported.
Poor mental health and well-being is also reported in the TES Wellbeing Report 2026[7] and showed that teachers are struggling to manage their wellbeing at work, with 57% of respondents reporting a moderate to negative overall wellbeing.

The Teacher Wellbeing Index summarises the impact succinctly as "...an unwell teaching workforce cannot deliver the best outcomes for children and young people. Quality of education is inseparable from the wellbeing of those who provide it".

Indicator	Figure	Source
Teachers reporting workplace stress in 2025	76%	Teacher Wellbeing Index 2025
At risk of probable clinical depression	36%	Teacher Wellbeing Index 2025
Experience symptoms of poor mental health due to work	77%	Teacher Wellbeing Index 2025
Proportion of staff that think that employees with mental health issues are not well supported	40%	Teacher Wellbeing Index 2025
Experiencing moderate to negative overall wellbeing	57%	Tes Teacher Wellbeing Report 2026

[6] https://www.educationsupport.org.uk/resources/for-organisations/research/teacher-wellbeing-index/
[7] https://www.tes.com/en-gb/for-schools/blog/article/state-teacher-wellbeing-schools

Blue Light Services

The people who are our blue light services also experience significant mental health related pressures. Four in five police officers experienced feelings of stress, low mood, anxiety, or other difficulties with their mental health or wellbeing in 2024. Work exacerbated these mental health issues[8]. More recently, the National Police Wellbeing survey showed that 45% of officers reported regularly feeling burnt out and 56% regularly feel exhausted by their work[9].

Earlier work from the University of Cambridge found that nearly all police work force had been exposed to trauma and one in five of the police workforce had experienced PTSD symptoms in the previous four weeks[10]. Although 73% of of the respondents had accessed mental health support only 16% of formal help had come from within the force.

This has impacts on staff retention. As part of the police federation 2024 survey members were asked what reasons they had for planning to leave the police service, and the mental health and wellbeing ramifications of the role had a major effect for 75 per cent.

Similar mental health issues also impact firefighters. 60% of fire service workers who took part in a 2019 survey by Mind, reported having personal experience of mental health problems.
The most commonly experienced mental health issues were depression (39.3%), anxiety (39.1%) and PTSD (18.1%)[11].

Mind note that while access to mental health support is improving for blue light services the demand keeps growing[12].

Indicator	Figure	Source
Police officers experiencing stress, low mood or anxiety	4 in 5	Police Federation 2024
Police officers reporting frequent burnout	45%	National Police Wellbeing Survey 2026
Police officers reporting regularly feeling physically exhausted by their work	56%	National Police Wellbeing Survey 2026

[8] https://polfed.org/our-work/wellbeing/mental-health/
[9] https://www.oscarkilo.org.uk/news/national-workforce-and-wellbeing-survey-results-published
[10] https://www.cam.ac.uk/stories/police-ptsd
[11] https://www.fbu.org.uk/magazine/december-2024-january-2025/mental-health-emergency
[12] https://www.mind.org.uk/news-campaigns/research-and-publications/

Firefighters with personal experience of a mental health problem	60%	Fire Brigades Union, Mind 2019
Firefighters self-reporting PTSD symptoms	18.1%	Fire Brigades Union, Mind 2019

Local government

The 2026 PPMA / HR Connect[13] survey shows that 26% of respondents in local government report feeling exhausted often or very often by the end of the day.

This is in part related to teams not having enough staff. Only 44% of local government respondents report having enough staff in their team. Not having enough staff translates to stress and 21% of local government respondents say staffing shortages create stress often or very often. The impact of this sustained pressure is reflected in levels of burnout and 'very low wellbeing'. 14% of local government respondents report they are very close to burnout or already burned out and 46% of respondents with not enough staff in their team report 'very low wellbeing'.

This level of mental health related issues is reflected in the 2024/25 TUC survey of health and safety representatives[14]. In local government stress was the top hazard identified by 66 % of respondents.

Indicator	Figure	Source
Feeling exhausted often or very often by the end of the day	26%	PPMA / HR Connect
Staffing shortage creating stress often or very often	21%	PPMA / HR Connect
Very close to burnout or already burned out	14%	PPMA / HR Connect
Of overwhelmed employees, those with 'very low wellbeing'	46%	PPMA / HR Connect
Local government safety reps citing stress as their top workplace concern	66%	TUC survey of health and safety representatives

[13] https://commercialservicesgroup.com/wellbeing-research/

[14] https://www.tuc.org.uk/sites/default/files/2025-12/SafetyRepsSurvey20242025.pdf

VCSE Sector

Mental health and stress impact the VCSE sector too. The 2024/25 TUC survey of health and safety representatives found that 71% of voluntary sector safety representatives cited stress as their top concern. This is in part reflected by the increasing demand on the voluntary sector to meet the support needs of the population. The VSCE barometer from Pro Bone Economics / Nottingham Trent University[15] shows over half of VCSEs expect increasing demand for support.

The increasing demand and workload is impacting staff with almost one in four (24%) citing stress and burnout as a direct result. This is increasing across a significant proportion of the voluntary sector with 1 in 3 charities reporting an increase in staff burnout and is also one of the main causes of retention issues[16].

Indicator	Figure	Source
Voluntary sector safety reps citing stress as their top concern	71%	TUC survey of health and safety representatives
Employees reporting an increased workload	70%	VSCE barometer
Of those, citing stress and burnout as a direct result	24%	VSCE barometer
Charities reporting an increase in staff burnout	Nearly 1 in 3	VSCE barometer

Across all the sectors we see similar patterns of stress and burnout affecting a significant proportion of the workforce, and the trend over the last few years is not improving. The TUC's 2025 survey found stress rated as the top workplace hazard in almost every sector and region, and is particularly impacting central and local government, health, education and the voluntary sector.

WholeMind Resilience can help close some of these gaps in support for our NHS workers, our educators, blue light workers, other public sector employees and our volunteers by providing accessible preventative mental health related support for staff. WholeMind Resilience can help give people the tools they need to proactively manage their mental well-being, reducing the likelihood of stress related absence or attrition.

[15] https://barometer.vcseobservatory.org.uk/en/trends/

[16] https://www.ntu.ac.uk/media/documents/academic-schools/nbs/2303-Running-hot,-burning-out.pdf

3. Vicarious trauma and compassion fatigue

Roles involving sustained exposure to others' distress carry a specific, additional risk beyond general workplace stress:

- Firefighters and paramedics: PTSD prevalence sits well above general population rates, driven by repeated exposure to traumatic incidents rather than workload alone.
- Police officers: overall PTSD rates in law enforcement are almost five times higher than general UK population levels and a majority feel they had insufficient time to process incidents
- Social workers and care staff: academic literature links sustained emotional and psychological strain in social work directly to compassion fatigue and secondary traumatisation, alongside higher turnover and absenteeism.
- NHS clinical staff: the 2025 NHS Staff Survey found more than a third of staff describe their work as emotionally exhausting, with burnout increasing specifically among staff with six or more years of service. This is consistent with cumulative, sustained exposure rather than a short-term pressure spike.

4. The legal and policy case, not just the moral one

Workforce mental wellbeing is not solely a matter of good practice. It sits within a specific, current legal and policy framework that gives employers a direct duty to act:

- The Equality Act 2010 requires employers to make reasonable adjustments for employees with mental health conditions that meet the legal definition of disability. This is a duty, not a discretionary benefit.
- The Health and Safety at Work etc. Act 1974, operationalised through the HSE's Management Standards, requires employers to risk-assess and proactively manage work-related stress across six specific areas: demands, control, support, relationships, role and change.
- The NHS Staff Standards (July 2026) make health and wellbeing support a compulsory, board-accountable, measured expectation for NHS employers. This is the first time this has been mandatory rather than advisory.

Services such as those provided by WholeMind Resilience are not a discretionary wellness perk sitting alongside statutory obligations. They are a practical route to meeting those statutory obligations. An organisation commissioning WholeMind Resilience is not just improving morale; it is directly supporting its own compliance with existing legal duties.

5. The cost of absence and presenteeism

As well as the personal and financial cost to employees there is the cost to employers and the wider economy. ACAS[17] highlights that almost a third of sickness absence includes stress, anxiety, depression and other mental health related issues. The Health and Safety Executive finds that

[17] https://www.acas.org.uk/a-third-of-workplace-sickness-absence-is-due-to-stress-anxiety-depression-or-other-mental-health

stress, depression or anxiety accounts for 22.1 million lost days[18]. And that on average each worker experiencing stress, depression or anxiety is absent for almost 23 days.

In terms of costs Deloitte estimate that the annual cost to UK employers of poor mental health is £51 billion[19]. Of which presenteeim (attending work while unwell) is £24 billion. Mental health related costs vary by age group with those aged 55 and over experience the lowest costs per employee.

Costs also vary by organisation. For example, in the NHS in April 2026, the almost 29% of absence due to mental health reasons resulted in 633,842 lost full time equivalent work days[20]. This has an impact on the ability of the NHS to meet the needs of patients and over the year costs the NHS about £2.4 billion. Presenteeism is consistently the largest single cost category and consistently under-discussed relative to absence: with over half of NHS staff attending work while unwell, the economic and safety case for prevention, not just treatment after the fact, is substantial.

Indicator	Figure	Source
Proportion of sickness absence including stress, anxiety and depression	32%	ACAS
UK working days lost to work-related stress, depression or anxiety	22.1 million days	HSE
Average length of absence for stress, depression or anxiety	22.9 days	HSE
Annual UK cost of poor mental health to employers (total)	£51 billion	Deloitte
Of which, presenteeism (the largest single driver)	£24 billion	Deloitte
Proportion of NHS sickness absence attributable to mental health	~29%	NHS Sickness Absence Rates
NHS staff sickness absence, annual cost	Estimated £2.4 billion	NHS Sickness Absence Rates

[18] https://www.hse.gov.uk/statistics/dayslost.htm

[19] https://www.deloitte.com/uk/en/services/consulting/research/mental-health-and-employers-the-case-for-employers-to-invest-in-supporting-working-parents-and-a-mentally-health-workplace.html

[20] https://digital.nhs.uk/data-and-information/publications/statistical/nhs-sickness-absence-rates

6. The economic case for prevention

Beyond the general cost of poor mental health, there is a clear, evidenced financial case for prevention and not just intervention in general. Deloitte estimate that the average return for every £1 invested in workplace mental health support is £4.70. However, this varies by type of intervention and universal preventative interventions return £6.30 for every £1 invested.

This aligns well with the WholeMind Resilience model for the delivery of universal group based preventative services, rather than restricting this to those already in crisis.

Indicator	Figure	Source
Average return for every £1 invested in workplace mental health support	£4.70	Deliotte
Return for every £1 invested specifically in universal, preventative interventions — available to all staff, before problems escalate	£6.30	Deliotte

7. NICE guidance and research: examples where the WholeMind Resilience model aligns

NICE guideline on mental wellbeing at work (NG212[21], 2022) is the definitive UK evidence-based standard for workplace mental health interventions. A number of NG212 recommendations map directly onto WholeMind's service model:

- **NG212, 1.2.1:** fostering a positive, compassionate and inclusive workplace environment. This maps to WholeMind Resilience's team trainings, Compassion-Based Reflective Practice offer, support for staff on phased return to work, and leadership specific services.
- **NG212, 1.6.4:** offer all employees (or help them to access) mindfulness, yoga or meditation on an ongoing basis. This can be delivered in a group or online, or using a combination of both. This maps to WholeMind's reflective practice services, and CMR delivery.
- **NG212, 1.8.1–1.8.3:** task-focused skills training for high-risk occupations, with good evidence of reduced mental health symptoms among police and healthcare professionals dealing with stressful occupational events. NICE cited Mind's Blue Light Programme as an example of best practice for high-risk occupational groups. However, that programme ended in July 2023. WholeMind Resileince can help fill the gap left by the closure of the programme and

[21] https://www.nice.org.uk/guidance/ng212

complement the national resources that are still available. For example, resources from the Blue Light Card Foundation[22].

- **NG212, rationale and impact:** the guideline committee agreed that "**mindfulness, meditation and yoga were the most effective interventions overall for reducing job stress and improving mental wellbeing."**

Other evidence available also shows how the mindfulness programme delivered by WholeMind Resilience can reduce stress and the likelihood of burnout. For example, the NIHR-funded 'Well at Work' study, recruited 260 staff across 13 NHS Trusts and compared an 8-week mindfulness-based course (MBCT-L) against a standard 4-week stress-reduction psychoeducation programme. Both are NICE-recommended interventions. However, the mindfulness course reduced stress significantly more effectively, improved depression, self-organisation and job satisfaction and reduced burnout.

For a comprehensive mapping of the WholeMind model to NICE guidelines NG212 and NG13[23], NICE quality statement [24] and the HSE Management Standards please see the companion Standards & Regulatory Alignment Report.

8. NHS Staff Standards and the NHS Staff Health and Wellbeing Framework: where the WholeMind model aligns

Both the NHS Staff Standards[25] and the NHS Health and Wellbeing Framework[26] align with the WholeMind Resilience model; prevention before crisis, proactive rather than reactive support, evidence-based and accountable delivery.

"NHS staff can't care for patients if they're not cared for themselves. The health and wellbeing standard sets clear expectations for employers."[27] NHS Staff Standards requires employers to take a proactive, evidence-based approach to protecting the physical and mental health of their workforce, going beyond reactive support to prevent harm before it happens, with clear accountability at board level.

WholeMind Resilience's offer will have an impact on some of the NHS staff survey questions designed to understand if an NHS organisation is delivering to the health and wellbeing standard in improvement in health and wellbeing for staff. For example, including through questions on whether staff:

- Feel their organisation takes positive action on health and wellbeing
- Have felt unwell in the past 12 months due to work-related stress
- Feel burnt out because of their work

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[22] https://bluelightcardfoundation.org/blue-light-together/

[23] https://www.nice.org.uk/guidance/ng13

[24] https://www.nice.org.uk/guidance/qs147

[25] https://www.gov.uk/government/publications/nhs-staff-standards/nhs-staff-standards-overview-and-summaries

[26] https://www.england.nhs.uk/supporting-our-nhs-people/health-and-wellbeing-programmes/nhs-health-and-wellbeing-framework/

[27] https://www.nhsemployers.org/staff-experience/nhs-staff-standards

The health and wellbeing standard sits alongside the NHS Health and Wellbeing Framework. The framework developed with wellbeing leads, HR directors and NHS staff themselves, provides toolkits and other resources for supporting staff to feel well, healthy and happy at work.

Part of the process of improving health and wellbeing outcomes for staff is giving line managers the resources to support the mental wellbeing of their staff. This will not only help achieve the NHS Staff Standards but will also help with staff retention. For example, surveys from Mind Forward Alliance[28] show employee desire to quit fell from 35% to 18% where line managers were well-equipped to have supportive mental health conversations, compared with employees whose managers were not.

WholeMind Resilience's Safe Leader Space is designed to give line managers the tools to support the mental wellbeing of their staff and help meet NHS staff standards.

9. Returning to work: how the WholeMind Resilience model supports a phased return approach

In different organisations patterns are similar. Once staff actually take time off for anxiety, depression, stress or burnout support when returning to work is inconsistent, and structured support is the exception rather than the rule.

The 2026 Mental Health UK, Burnout Report[29] found that one in four (27%) workers who took time off due to extreme levels of pressure or stress said they did not receive any support after they had returned to work, and only 17% had a formal return-to-work plan put in place. This is despite the time of returning to work being one of the periods with the highest risk of relapse and subsequent further absence.

Even if a workplace has good intentions the report also found that there is a gap between these good intentions and concrete efforts. Only 32% of workplaces had formal plans and processes to identify chronic stress before burnout, almost one in three workers (29%) said their employer raises awareness about mental health but managers lack the time, training and resources to provide meaningful support and only 11% had regular check-in meetings with their manager after returning.

WholeMind Resilience's CMR for Phased Return is designed to help close this high risk gap in an organisations mental health support offer and reduce the risk of repeated absence through its structured, evidence informed phased return programme.

The WholeMind offer meets the recommendations from NICE NG13 to avoid return to work processes that encourage presenteeism and supports the NHS Staff Standards to provide access to occupational health and wellbeing support and act on recommendations.

Indicator	Figure	Source
Workers who received no support at all on return from stress-related absence	27%	2026 Mental Health UK, Burnout Report

[28] https://mindforwardalliance.com/Resources/Global-Research-Study-Report#doc-1199

[29] https://mentalhealth-uk.org/burnout/

Had a formal return-to-work or burnout recovery plan put in place	17%	2026 Mental Health UK, Burnout Report
Workplaces with any formal plan to identify chronic stress before it becomes burnout	32%	2026 Mental Health UK, Burnout Report
Employer raises awareness but managers lack the time, training and resources to provide meaningful support	29%	2026 Mental Health UK, Burnout Report
Had regular wellbeing check-in meetings with their manager after returning	11%	2026 Mental Health UK, Burnout Report

10. What this shows: a consistent pattern that WholeMind Resilience can help address

This report brings together survey information from different organisations and sectors. One of the most striking features is the consistency of the patterns of need for support. Whether the workforce is nurses, teachers, police officers, care workers, charity staff or other organisations, the underlying pattern is the same:

- For workers, sustained (and increasing) emotional and operational demand, insufficient preventative support and rising burnout.
- For employers, a direct financial and staff cost through absence and presenteeism.

The WholeMind Resilience service model is designed to address these challenges:

- Accessible preventative mental health related support for staff to help give people the tools they need to proactively manage their mental well-being, reducing the likelihood of stress related absence or attrition.
- CMR for Phased Return designed to help close this high risk gap in an organisations mental health support offer and reduce the risk of repeated absence through its structured, evidence informed phased return programme.
- A Safe Leader Space is designed to give line managers the tools to support the mental wellbeing of their staff.

Providing these services for staff will improve staff wellbeing and retention, reduce costs due to absence and presenteeism, support an organisation's existing legal duties under the Equality Act 2010 and Health and Safety at Work Act 1974 (not just its wellbeing ambitions), and, in the case of the NHS, help meet the NHS Staff Standards of the requirement for employers to take a proactive, evidence-based approach to protecting the physical and mental health of their workforce.

WholeMind is a cross-sector prevention partner, delivering research backed services designed to fit the needs of organisations. Its core service model is the approach recommended by NICE and includes the most effective interventions overall for reducing job stress and improving mental wellbeing.

Appendix:

- NHS Staff Survey 2025 (NHS Employers / NHS Confederation, published March 2026) — 766,285 responses.
- NHS England Digital, NHS Sickness Absence Rates, April 2026.
- The Care Worker's Charity, Wellbeing Report 2025.
- Care Quality Commission, State of Care Report 2023/24.
- The King's Fund, Social Care 360: Workforce and Carers (2026), citing Skills for Care workforce data.
- Education Support, Teacher Wellbeing Index 2025 (ninth annual survey).
- Tes, Teacher Wellbeing Report 2026.
- Police Federation of England and Wales, wellbeing data 2024/25.
- National Police Wellbeing Service (Oscar Kilo), National Police Wellbeing Survey 2025 (40,000+ respondents, 33 forces).
- Fire Brigades Union / Mind, cited in Firefighter Magazine, 2024–2025.
- Public Services People Managers Association (PPMA) / HR Connect, public sector wellbeing survey, 2026.
- Trades Union Congress (TUC), biennial safety representatives survey, 2025.
- Pro Bono Economics / Nottingham Trent University, VCSE Barometer, 'Running Hot, Burning Out'.
- Deloitte, Mental health and employers: the case for investment (4th edition, 2024).
- Health and Safety Executive, Health and Safety at Work statistics 2024/25, and Management Standards for work-related stress.
- NICE NG212, Mental wellbeing at work (2022).
- NICE NG13, Workplace health: management practices (2015, updated 2016).
- NICE QS147, Healthy workplaces: improving employee mental and physical health and wellbeing (2017).
- Department of Health and Social Care / NHS England, NHS Staff Standards (published 6 July 2026).
- NHS Employers, NHS Health and Wellbeing Framework (2024).
- NIHR, 'Well at Work' study — MBCT-L trial across 13 NHS Trusts.
- Mental Health UK, Burnout Report 2026 (YouGov survey of 4,502 UK adults, fieldwork November 2025).
- The Global Business Collaboration for Better Workplace Mental Health, manager training and staff retention research, cited via MHFA England and Management Today.
- Equality Act 2010; Health and Safety at Work etc. Act 1974.
- WholeMind Resilience CIC, LinkedIn poll of public service professionals, July 2026 (122 respondents).
- WholeMind Resilience CIC, Standards & Regulatory Alignment Report (companion document, full NICE/NHS/HSE criteria mapping).